



File No.: EXP202404257

RESOLUTION OF RIGHTS PROCEDURE

Having reviewed the complaint registered on February 27, 2024, with this Agency and having carried out the procedural actions provided for in Title VIII of Organic Law 3/2018, of December 5, on the Protection of Personal Data and the Guarantee of Digital Rights (hereinafter LOPDGDD), the following FACTS have been established:

FIRST: On February 27, 2024, a complaint was received by this Agency filed by Ms. A.A.A. (hereinafter, the complainant) against SERVICIOS DE FISIOTERAPIA, S.A. (hereinafter, the respondent) for not having duly attended to her right of access to her medical history.

The complainant states that "(...) access to my medical history, as well as a complete copy of it, has been requested on numerous occasions, both verbally and in writing, from Corporació Fisio Gestió (trade name), Servicios de Fisioterapia, S.A., without my request being attended to despite my insistence."

She indicates that, as a result, she sought legal advice to request the medical history on her behalf, which occurred via a burofax dated November 7, 2023, without receiving a response from the respondent.

She attaches a copy of said burofax in which it is stated that "I, B.B.B. (...) am addressing you in my capacity as a specialist lawyer and acting on behalf of Ms. A.A.A., with ID number (...), in order to request a complete copy of the medical history held in your files, in accordance with the applicable legal provisions (...)"

In light of all the above, I request that the copy of the Medical History be provided to me as soon as possible and, in any case, within a period not exceeding seven calendar days from the notification of this document, in accordance with current legislation (you may send the documentation to the email address provided at the end of this page) (...)"

Furthermore, the complainant requests that "(...) it be resolved, if deemed appropriate, the granting of compensation or economic reparation quantified at two thousand five hundred sixty-eight euros with seventy cents (€2,568.70) for the violation of data protection and GDPR, for not being guaranteed my right of access to my medical history."

SECOND: In accordance with Article 65.4 of the LOPDGDD, this complaint was forwarded to the respondent, so that they could analyze it and inform this Agency within one month of the actions taken to comply with the requirements set forth in data protection regulations.

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28001 – Madrid

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It is not recorded that the respondent has submitted any response.

THIRD: The result of the transfer procedure indicated in the previous Fact did not allow for the understanding that the claims of the claimant were satisfied. Consequently, on May 27, 2024, for the purposes provided in Article 64.1 of the LOPDGDD, the Director of the Spanish Agency for Data Protection agreed to admit the complaint filed.

The aforementioned agreement granted the respondent a hearing procedure, so that within ten working days, they could present the arguments they deemed appropriate.

The respondent stated: "(...) That, indeed, this party received a burofax requesting a complete copy of the medical history of Ms. A.A.A. The aforementioned request was made exclusively by attorney B.B.B.

That Ms. A.A.A. has been a patient of the entity I represent and, therefore, this party has the requested documentation, but a response could not be given to the request since the requester was not the patient herself and the documentation presented did not include any type of authorization, consent, or identity document of the rights holder that would allow the aforementioned attorney to act as a

representative and authorize a positive response to the request. The mere request along with the basic data of the patient was considered insufficient to prove the necessary representation to access such sensitive documentation, resulting from a personal right, and that in application of the current legal regulations, it was not possible to provide a positive response to the request.

(...) That this party does not place any impediment in complying with what is established in the current regulations, in this case, Law 41/2002 of November 14 in its Article 18, and in light of the content of the notification issued by this Agency, we have understood that Ms. A.A.A. requires her medical history. For this reason, we have sent it to her private address on July 24, 2024 (We attach a copy of the burofax sent as an annexed document). Thus, we comply with her request, delivering the documentation to the person who holds the real legitimacy to request it, not to someone who requests it without proving any right.”

FOURTH: After examining the document submitted by the respondent, it is transferred to the claimant, so that, within ten working days, they may formulate the arguments they consider appropriate, stating, in summary, that as they expressed in their complaint, they requested access to their medical history and a copy of it on numerous occasions verbally, without obtaining results.

Therefore, they hired an attorney to request the medical history on their behalf.

“The responsible entity finally sent me a document that claims to be my medical history. However, this document is manifestly insufficient, as it is a partial, incomplete, and extremely brief history. It lacks essential information that, according to current health regulations, must be included in a complete and detailed medical history. Among the omitted data are...

C/ Jorge Juan, 6

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28001 – Madrid

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3/12

key elements such as complete reports, documents, and medical information necessary to continue with my treatment. (...)”

In her written allegations, the claimant requests that “(...) it be resolved, if deemed appropriate, to grant compensation or economic reparation for the violation of data protection and GDPR, for not guaranteeing my right of access to my medical history.”

FIFTH: After granting a hearing to the defendant, it states “It is an undisputed fact that this party has sent Ms. A.A.A. her clinical documentation and she has received it, therefore we have complied with what is established by the regulations regarding patient access to it.”

“(...) Ms. A.A.A. insists that this party did not respond to the request for her documentation made through a lawyer. This party certifies the truthfulness of her statement, but Ms. A.A.A. omits that Law 41/2002 of November 14 in its article 18 establishes that representation must be duly accredited, a circumstance that did not occur in this case. As this party already argued in its allegations dated 26.07.24, the delay in delivering the documentation was due to the fact that the request for documentation via burofax was made by a lawyer without this person providing the necessary accreditation to access such sensitive documentation, resulting from a personal right, and therefore it was not answered. Furthermore, this party is not aware that Ms. A.A.A. requested the aforementioned documentation through any other channel.”

“(...) Ms. A.A.A. states that the content of her medical history is “partial, incomplete, and extremely brief.” Regardless of the party's assessments, the content of each patient's medical history is established by article 15 of Law 41/2002 of November 14, which regulates the autonomy of the patient and rights and obligations regarding information and clinical documentation. The cited provision details the content of the documentation that, under medical criteria, allows for a truthful and updated understanding of the patient's health status. In the clinical documentation that this party sent to Ms. A.A.A., the following is included: clinical statistical documentation, anamnesis and physical examination, pathological history, diagnosis, treatment, evolutionary course, and reason for discharge. A discharge report was also provided. Given that we are talking about a physical rehabilitation process in which written informed consent is not required, there is no surgery, no nursing intervention, nor consultations or complementary tests in the possession of this party, we consider that the

documentation delivered to Ms. A.A.A. fully complies with legal requirements and has been prepared under medical criteria (...) Another issue is that what is transcribed in the documentation is not to the applicant's liking, (...)"

LEGAL GROUNDS

I

Competence

In accordance with the powers granted by Article 58.2 of Regulation (EU) 2016/679 (General Data Protection Regulation, hereinafter GDPR), to each

C/ Jorge Juan, 6

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28001 – Madrid

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4/12

control authority and according to the provisions established in articles 47, 48.1, and 64.1 of the LOPDGDD, the Director of the Spanish Agency for Data Protection is competent to initiate and resolve this procedure.

Furthermore, article 63.2 of the LOPDGDD states that: "The procedures processed by the Spanish Agency for Data Protection shall be governed by the provisions of Regulation (EU) 2016/679, by this organic law, by the regulatory provisions issued for its development, and, in so far as they do not contradict them, subsidiarily by the general rules on administrative procedures."

II

Preliminary Issues

In accordance with the provisions established in article 55 of the GDPR, the Spanish Agency for Data Protection is competent to perform the functions assigned to it in article 57, including the enforcement of the Regulation and promoting awareness among data controllers and processors regarding their obligations, as well as addressing complaints filed by a data subject and investigating, as appropriate, the reasons for such complaints.

Correspondingly, article 31 of the GDPR establishes the obligation of data controllers and processors to cooperate with the control authority that requests it in the performance of its functions. In the case that they have appointed a data protection officer, article 39 of the GDPR assigns this officer the function of cooperating with said authority.

In accordance with this regulation, following the procedure provided in article 65.4 of the LOPDGDD, prior to the admission of the complaint regarding the exercise of the rights regulated in articles 15 to 22 of the GDPR, which gives rise to this procedure, the complaint was forwarded to the respondent party for analysis, to respond to this Agency within one month, and to demonstrate that they had provided the due response to the complainant.

The result of this forwarding did not allow for the understanding that the claims of the complainant were satisfied. Consequently, on May 27, 2024, for the purposes provided in articles 64.1 and 65 of the LOPDGDD, the complaint was admitted for processing. This admission for processing determines the opening of the present procedure for the lack of attention to a request for the exercise of the rights established in articles 15 to 22 of the GDPR, regulated in article 64.1 of the LOPDGDD, according to which:

"When the procedure refers exclusively to the lack of attention to a request for the exercise of the rights established in articles 15 to 22 of Regulation (EU) 2016/679 of the European Parliament and of the Council, of April 27, 2016, it shall be initiated by an admission agreement, which shall be adopted in accordance with the provisions of article 65 of this organic law.

In this case, the deadline for resolving the procedure shall be six months from the date on which the admission agreement was notified to the complainant. After this period, the interested party may consider their complaint accepted."

C/ Jorge Juan, 6

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28001 – Madrid

Article 58.2 of the GDPR grants the Spanish Data Protection Agency a series of corrective powers for the purpose of rectifying any infringement of the GDPR, including the power to "order the controller or processor to address requests for the exercise of the rights of the data subject under this Regulation."

III

Rights of Individuals Regarding Personal Data Protection

The rights of individuals regarding personal data protection are regulated in Articles 15 to 22 of the GDPR and Articles 13 to 18 of the LOPDGDD. These include the rights of access, rectification, erasure, objection, the right to restriction of processing, and the right to data portability.

The formal aspects related to the exercise of these rights are established in Articles 12 of the GDPR and 12 of the LOPDGDD.

Additionally, consideration is given to the provisions expressed in Recitals 59 and following of the GDPR.

In accordance with the provisions of these regulations, the data controller must establish means and mechanisms to facilitate the exercise of rights by the data subject, which shall be free of charge (without prejudice to the provisions of Articles 12.5 and 15.3 of the GDPR), and is obliged to respond to requests made no later than one month, unless it can demonstrate that it is unable to identify the data subject, and must state its reasons if it does not intend to comply with such request. The burden of proof for compliance with the obligation to respond to the request for the exercise of rights made by the affected party lies with the controller.

The communication addressed to the data subject regarding their request must be expressed in a concise, transparent, intelligible, and easily accessible manner, using clear and simple language.

IV

Right of Access

In accordance with the provisions of Article 15 of the GDPR and Article 13 of the LOPDGDD, "the data subject has the right to obtain from the data controller confirmation as to whether or not personal data concerning them is being processed and, if so, the right of access to the personal data."

Regarding the right of access to personal data, according to the provisions of Article 13 of the LOPDGDD, when the exercise of the right refers to a large amount of data, the controller may request the affected party to specify the "data or processing activities to which the request refers." The right shall be deemed granted if the controller provides remote access to the data, and the request shall be considered fulfilled (although the data subject may request the information referred to in the aspects provided in Article 15 of the GDPR).

C/ Jorge Juan, 6

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28001 – Madrid

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The exercise of this right may be considered repetitive if it is exercised more than once within a period of six months, unless there is a legitimate reason for doing so. On the other hand, the request will be considered excessive when the affected party chooses a means other than the one offered that entails a disproportionate cost, which must be borne by the affected party.

Like the other rights of the interested party, the right of access is a personal right. It allows the citizen to obtain information about the processing of their data, the possibility of obtaining a copy of the personal data concerning them that is being processed, as well as information, in particular, about the purposes of the processing, the categories of personal data concerned, the recipients or categories of recipients to whom the data may be communicated, the expected retention period or criteria for retention, the possibility of exercising other rights, the right to lodge a complaint with the supervisory authority, the information available about the source of the data (if it has not been obtained directly from the data subject), the existence of automated decisions, including profiling, and information about transfers of personal data to a third country or to an international organization. The possibility of

obtaining a copy of the personal data being processed shall not negatively affect the rights and freedoms of others.

The right of access in relation to the medical history is specifically regulated in Article 18 of Law 41/2002, of November 14, Basic Law Regulating Patient Autonomy and Rights and Obligations in the Field of Information and Clinical Documentation (hereinafter LAP), the literal text of which states:

"1. The patient has the right of access, with the reservations indicated in section 3 of this article, to the documentation of the medical history and to obtain a copy of the data contained therein. Health centers shall regulate the procedure that guarantees the observance of these rights.

2. The patient's right of access to the medical history may also be exercised by duly accredited representation.

3. The patient's right of access to the documentation of the medical history may not be exercised to the detriment of the right of third parties to the confidentiality of the data contained therein collected in the therapeutic interest of the patient, nor to the detriment of the rights of the professionals involved in its preparation, who may oppose the right of access the confidentiality of their subjective notes.

4. Health centers and individual practitioners shall only facilitate access to the medical history of deceased patients to persons linked to them, for family reasons or, in fact, unless the deceased expressly prohibited it and this is accredited. In any case, access by a third party to the medical history motivated by a risk to their health shall be limited to the relevant data. Information affecting the privacy of the deceased or the subjective notes of the professionals, or that harms third parties, shall not be provided."

C/ Jorge Juan, 6
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28001 – Madrid
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7/12

In this regard, it is important to highlight Article 15 of the LAP, which outlines the minimum content of the medical history:

"1. The medical history shall incorporate the information deemed essential for the truthful and updated knowledge of the patient's health status. Every patient or user has the right to have a record, in writing or in the most appropriate technical support, of the information obtained in all their care processes, carried out by the health service both in the primary care and specialized care settings.

2. The primary purpose of the medical history shall be to facilitate healthcare, recording all those data that, in the medical opinion, allow for the truthful and updated knowledge of the health status.

The minimum content of the medical history shall be as follows:

- a) The documentation related to the clinical-statistical sheet.
- b) The admission authorization.
- c) The emergency report.
- d) The anamnesis and physical examination.
- e) The evolution.
- f) The medical orders.
- g) The consultation sheet.
- h) The reports of complementary examinations.
- i) The informed consent.
- j) The anesthesia report.
- k) The operating room report or delivery record.
- l) The pathology report.
- m) The evolution and planning of nursing care.
- n) The therapeutic application of nursing.
- ñ) The vital signs chart.
- o) The clinical discharge report.

Paragraphs b), c), i), j), k), l), ñ), and o) shall only be required in the completion of the medical history when it concerns hospitalization processes or as provided.

3. The completion of the medical history, in aspects related to direct patient care, shall be the responsibility of the professionals involved in it.

4. The medical history shall be maintained with criteria of unity and integration, in each healthcare institution at a minimum, to facilitate the best and most timely knowledge by the professionals of the data of a particular patient in each care process."

Regarding the preservation of the medical history, Article 17 of the LAP, in its points 1 and 5, states that:

"1. Health centers have the obligation to preserve clinical documentation under conditions that guarantee its proper maintenance and security, even if not...

C/ Jorge Juan, 6

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28001 – Madrid

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8/12

necessarily in the original format, for the proper assistance to the patient during the appropriate time for each case and, at a minimum, five years counted from the date of discharge of each care process (...)

5. Healthcare professionals who carry out their activity individually are responsible for the management and custody of the care documentation they generate."

V

Conclusion

In the case analyzed here, the claiming party stated that they had requested the right of access to their medical history through a representative, sending a burofax to the responding party, without receiving a reply.

During the instruction of this procedure, the responding party indicated that the right of access was requested by a person claiming to act on behalf of the claiming party but without presenting documentation regarding such representation, which is why they did not respond, given that the right is personal.

Notwithstanding the above, after receiving the complaint forwarded by this Agency, they have sent a copy of the medical history directly to the claiming party's address.

The claiming party has expressed their disagreement with the documentation received, indicating that it is incomplete, and that the medical history received does not meet the requirements established in Law 41/2002, lacking various documents.

The Guidelines 1/2022, on the rights of data subjects — Right of access Version 2.1 of March 28, 2023, from the European Data Protection Committee, state that

"3.4 Requests made through third parties/representatives

80. Although the right of access is generally exercised by the data subjects concerning their own matters, it is possible for a third party to submit a request on behalf of the data subject. This may apply, among other things, to acting through a representative or legal guardians on behalf of minors, as well as to acting through other entities via online portals. In some circumstances, the identity of the person authorized to exercise the right of access, as well as the authorization to act on behalf of the data subject, may require verification, provided that it is appropriate and proportionate (see section 3.3). It should be noted that providing personal data to someone who does not have the right to access it may constitute a violation of personal data security.

81. In doing so, national legislation regulating legal representation (for example, powers of representation) must be taken into account, which may impose specific requirements to demonstrate the authorization to submit a request on behalf of the data subject, as the GDPR does not regulate this issue. In accordance with the principle of proactive responsibility, as well as with the other principles of the...

C/ Jorge Juan, 6

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28001 – Madrid

data protection, the data controllers must be able to demonstrate the existence of the relevant authorization to submit a request on behalf of the data subject and to receive the requested information, unless national legislation differs (for example, national legislation contains specific rules regarding the reliability of lawyers), leaving it to the data controller to verify the identity of the representative (for example, in the case of lawyers, by checking their registration with the bar association). Therefore, it is recommended to collect the appropriate documentation in this regard, in relation to the general rules previously indicated regarding the confirmation of the identity of a natural person submitting a request and, if the data controller has reasonable doubts about the identity of a person acting on behalf of the data subject, they will request additional information to confirm the identity of that person.” (The underlining is ours)

The aforementioned Guidelines 1/2022 also indicate:

“3 GENERAL CONSIDERATIONS REGARDING THE EVALUATION OF ACCESS REQUESTS

3.1 Introduction

41. Upon receiving requests for access to personal data, the data controller must evaluate each request individually. The data controller will take into account, among other things, the following issues, which are developed in the subsequent sections: whether the request pertains to personal data linked to the requesting person and who the requesting person is. The aim of this section is to clarify which elements of the access request the data controller must consider when conducting their evaluation and to discuss the possible scenarios of that evaluation, as well as its consequences. When evaluating a request for access to personal data, the data controller will also take into account, in accordance with Article 12, paragraph 2, of the GDPR, the obligation to facilitate the exercise of the rights of the data subject, while also considering the appropriate security of personal data.

42. Therefore, data controllers must be proactively prepared to process requests for access to personal data. This means that the data controller must be ready to receive the request, adequately evaluate it (this evaluation is the subject of this section of the guidelines), and provide an appropriate response without undue delay to the requesting person. The manner in which data controllers prepare for the exercise of access requests must be appropriate and proportionate and depend on the nature, scope, context, and purposes of the processing, as well as the risks to the rights and freedoms of natural persons, in accordance with Article 24 of the GDPR. Depending on the particular circumstances, data controllers may be required, for example, to apply an appropriate procedure, the implementation of which must ensure the security of the data without hindering the exercise of the rights of the data subject.” (The underlining is ours)

In the present case, upon examining the documentation in the file, it is observed that, as indicated by the responding party, the request for access to the history

C/ Jorge Juan, 6

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28001 – Madrid

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The clinic submitted by its legal representative lacked the legal requirements to validate such representation, which means that the right could not be considered validly exercised. However, there is no record that the responding party addressed this request indicating such circumstance and requesting its correction.

Notwithstanding the above, during the instruction of the present procedure, the responding party has directly sent to the claiming party, not to the person who stated they were acting on their behalf, a copy of their medical history.

The aforementioned rules do not allow for the request to be overlooked as if it had not been made, leaving it without the response that those responsible must necessarily issue, even in the event that there are no data of the interested party in the entity's files or even in those cases where the requirements set forth are not met, in which case the recipient of such request is also obliged to require the correction of the observed deficiencies or, if applicable, deny the request with justification

indicating the reasons why the right in question cannot be considered.

Therefore, the request made obliges the responsible party to provide an express response, in any case, using any means that justifies the receipt of the response.

In this case, however, it is noted that during the processing of the claim that gives rise to the present procedure, formulated precisely due to the lack of an adequate response to the request for the exercise of rights, the responding party has provided documentation proving the communication sent to the interested party regarding the right or informing about the decision made regarding the request. This response, therefore, occurs after the established deadline has passed.

In this regard, it should be specified that the response that corresponds to be made cannot be expressed merely as part of an administrative procedure. Consequently, considering that the present procedure aims to ensure that the guarantees and rights of the affected parties are duly restored, combining the information in the file with the regulations referred to in the preceding sections, it is appropriate to uphold this claim on formal grounds due to the response being issued late, without requiring the responsible party to carry out additional actions in order to issue a new certification regarding the attention to the right.

Regarding the disagreement of the claiming party indicating that the medical history is incomplete, according to the provisions of Law 41/2002, it should be noted that this Agency does not possess sufficient information to analyze and assess whether that medical history is complete or not. The responding party has stated that the medical history submitted consists of clinical-statistical documentation, anamnesis and physical examination, pathological history, diagnosis, treatment, evolutionary course, discharge report, and reason for discharge. According to Article 15 of the Law, this documentation is part of the minimum content, unless it pertains to a hospitalization, a fact that the responding party has not substantiated or stated. Therefore, in the absence of further information from this authority regarding the pathology, it is understood that the medical history submitted by the responding party meets the legal requirements.

C/ Jorge Juan, 6
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28001 – Madrid
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11/12

On the other hand, regarding the compensation request made, the claiming party must address the ordinary jurisdictional bodies, in accordance with the provisions of Article 82, paragraphs 1 and 6, in relation to Article 79.2 of the GDPR, which establishes the following:

Article 79.2

2. Actions against a controller or processor must be brought before the courts of the Member State in which the controller or processor has an establishment. Alternatively, such actions may be brought before the courts of the Member State in which the data subject has their habitual residence, unless the controller or processor is a public authority of a Member State acting in the exercise of its public powers.

Article 82.1 and 6

1. Any person who has suffered material or immaterial damage as a result of an infringement of this Regulation shall have the right to receive compensation from the controller or processor for the damage suffered.

(...)

6. Judicial actions exercising the right to compensation shall be brought before the competent courts in accordance with the law of the Member State indicated in Article 79, paragraph 2.

Furthermore, recitals 146 and 147 of the GDPR state that

“(146) The controller or processor must compensate for any damage that a person may suffer as a result of processing in violation of this Regulation. The controller or processor shall be exempt from liability if it is proven that they are not responsible for the damage. The concept of damage must be interpreted broadly in light of the case law of the Court of Justice, so that the objectives of this Regulation are fully respected. This is understood without prejudice to any claims for damages arising from the infringement of other rules of Union or Member State law. Processing in violation of this

Regulation also includes processing that infringes delegated acts and implementing acts adopted in accordance with this Regulation and the law of the Member States that specify the rules of this Regulation. Data subjects must receive full and effective compensation for the damage suffered. If the controllers or processors participate in the same processing, each controller or processor shall be considered liable for the entirety of the damage. However, if they are joined in the same cause in accordance with the law of the Member States, compensation may be apportioned according to the liability of each controller or processor for the damage caused by the processing, provided that the total and effective compensation of the data subject who suffered the damage is guaranteed. Any controller or processor that has paid the full compensation may subsequently seek recourse against other controllers or processors that participated in the same processing.

(147) In cases where this Regulation contains specific rules on jurisdiction, particularly regarding actions seeking judicial redress, including compensation, against a controller or processor, the general rules of jurisdiction as established in Regulation (EU) No. 1215/2012 of the European Parliament and of the Council (1) must be understood without prejudice to the application of such specific rules.”

In view of the cited provisions and other generally applicable ones, the Director of the Spanish Data Protection Agency RESOLVES:

FIRST: TO ACCEPT for formal reasons, the claim made by Ms. A.A.A. against SERVICIOS DE FISIOTERAPIA, S.A. However, the issuance of a new certification by the respondent is not warranted, as the response was issued late, and no additional actions are required from the controller.

SECOND: TO NOTIFY this resolution to Ms. A.A.A. and to SERVICIOS DE FISIOTERAPIA, S.A.

In accordance with the provisions of Article 50 of the LOPDGDD, this Resolution will be made public once it has been notified to the interested parties.

Against this resolution, which concludes the administrative procedure in accordance with Article 48.6 of the LOPDGDD, and in accordance with the provisions of Article 123 of the LPACAP, the interested parties may optionally file a motion for reconsideration before the Director of the Spanish Data Protection Agency within one month from the day following the notification of this resolution or directly file an administrative contentious appeal before the Administrative Chamber of the National Court, in accordance with the provisions of Article 25 and paragraph 5 of the fourth additional provision of Law 29/1998, of July 13, regulating the Administrative Jurisdiction, within two months from the day following the notification of this act, as provided in Article 46.1 of the aforementioned Law.

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